National Cancer Control Programme (NCCP) 00712 · clinician draft

Nivolumab, ipilimumab, paclitaxel and carboplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Active medicines: nivolumab (Days 1 and 22) + ipilimumab (Day 1 only) + paclitaxel (Days 1 and 22) + carboplatin (Days 1 and 22)

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Repeat the three-day course separately around Cycle 1 Day 1 and Day 22
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Repeat around both chemotherapy visits The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.
Paclitaxel dexamethasone premedication	Source reference 20 mg oral or intravenous (IV)	Oral approximately 6 and 12 hours before paclitaxel, or IV 30 minutes before Reconcile with antiemetic steroid; source advises considering 12 mg on treatment day when aprepitant is added. Consultant may reduce/omit only under explicit guidance.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Omission requires consultant guidance. Source cautions particularly with every-three-week paclitaxel; monitor closely for hypersensitivity.

Additional phase-specific support

- Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1.
- Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.
- The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
- Use dedicated carboplatin AUC≥4 NCIS row; no routine olanzapine in that row. Myalgia/arthralgia may need an individual analgesia plan.
- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00712 v3a · pNCCP 00712 pp8–9; NCIS V8 Table 1 p4 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Cycle 2 onwards · Day 1 nivolumab + ipilimumab

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.

Medicine	Dose / route	When
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.
- Chemotherapy is complete; review and stop induction prophylaxis rather than automatically repeating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00712 v3a · pNCCP 00712 pp8–9; NCIS V8 Table 1 p4 · NCIS low-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Cycle 2 onwards · Day 22 nivolumab alone

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00712 v3a · pNCCP 00712 pp8–9; NCIS V8 Table 1 p4 · Current NCCP regimen

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required